Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

PERIOPERATIVE CARE

Anaesthesia for inpatient Hip & Knee Arthroplasties

Enhanced Recovery After Surgery
Helping you get better, quicker

YG CG 412 SEPT 2021

REVIEW BY SEPT 2024

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Reviewer: Anaesthetic Department

It is the responsibility of the prescriber to ensure that there are no contraindications to the following:

Premed:

Longtec® (Oxycodone M/R) PO dose range: 5-10mg PO depending on age / comorbidity

Clear fluids until 2 hours pre-operative then sips of water.

Anaesthetic:

GA or Spinal according to patient's preference / medical indication.

For spinal anaesthesia - 15-25mcg of intrathecal Fentanyl can be considered.

Avoid intrathecal Diamorphine in uncomplicated arthroplasty.

300mcg Diamorphine for revision arthroplasty, bilateral arthroplasty, complex cases, chronic pain patients.

Pre-incision:

Antibiotics (IV Cefuroxime 1.5g plus IV Gentamicin <60Kg: 160mg, >60Kg: 240mg on induction of anaesthesia)
- In severe penicillin allergy or current/previous MRSA: IV Teicoplanin (<50Kg: 600mg, >50Kg: 800mg) plus Gentamicin

Dexamethasone 9.9mg IV (dose range 8-10mg)

Tranexamic Acid 1g IV (or 10mg/kg)

Intra-op:

<u>For knee replacement surgery</u> – Adductor canal block should be performed for both spinal and general anaesthesia.

Local anaesthetic infiltration by surgeons – Standard mix = 3 x 50 mls syringes: each containing 20mls of 0.25% bupivicaine with adrenaline plus 30mls saline *(if patient <55kg use same mix but half volume. 55-74kg use same mix but discard 12mls from each syringe)*

Paracetamol 1g (or 15mg/kg if <50kg)

NSAID if no contraindications - IV diclofenac 75mg

Magnesium Sulphate 5g (20mmol) infusion during surgery.

Ondansetron 4mg IV

<u>For GA's:</u>

Fentanyl in preference to Morphine (500mcg/10ml Fentanyl is available in theatre recovery)

(+/- consider catheter in joint for post op infiltration – e.g. for patients on long term opioids. Up to 24 hours maximum)

Inpatient drug chart – Prescribe:

Recovery:

1. Fentanyl 5 x 20mcg boluses for break through pain in recovery

Regular:

2. Paracetamol 1g QDS PO (15mg/kg if <50kg) *(cross off any doses given in theatre)*

3. Ibuprofen 400mg TDS PO *(cross off any doses given in theatre)*
   - *Celecoxib 200mg BD PO if hx of gastric ulcer/dyspepsia*
   - *Reduce dose and duration in elderly or frail.*
   - *Avoid completely <u>only</u> if hypersensitive or eGFR <30ml/min – see chart →*

4. Omeprazole 20mg OD PO

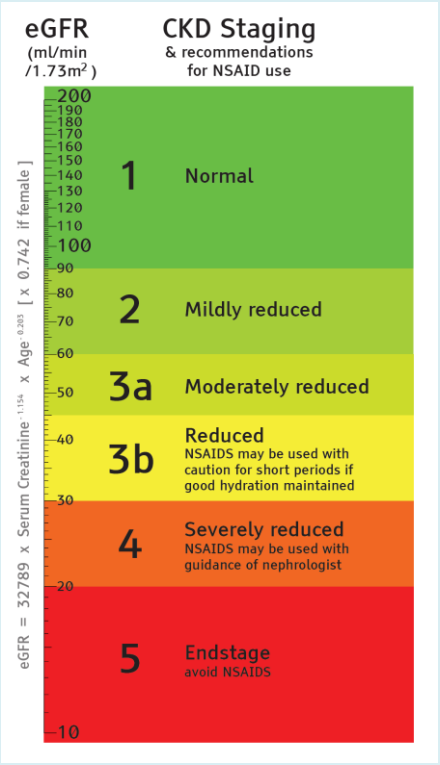
5. Longtec® (Oxycodone M/R) 5-10mg BD PO *(next dose will be at 8pm)*

6. Ondansetron 4mg TDS PO

7. Lactulose 15ml BD PO

8. Senna 15mg Nocte PO

9. Enoxaparin 40mg s/c NOCTE
   - Reduce to 20mg if <50kg or CrCl<30ml/min
   - Consider increasing Enoxaparin dose to 60mg if >110kg (unlicensed)



PRN:

10. Shortec® (Oxycodone) 5-10mg PO (Max 2 Hourly) PRN

11. Naloxone 100-400mcg IV PRN

12. Cyclizine 50mg PO/IV TDS PRN

Tudno ward:* *Call Acute Pain team (bleep 115) or Anaesthetist (063) if pain needs reviewing urgently